Service Specification No.	
Service	**New Models Programme- Neighbourhood Access Service**
Commissioner Lead	**Ellie Wagg**
Provider Lead	
Period	
Date for Review	

1. Population Needs

National/Local Context and Evidence Base

The New Models Programme is one part of the Integrated Care Board's(ICB) response to Delivering an Integrated Neighbourhood Health Service. It is a long-term transformation programme that will bring together integrated Neighbourhood teams to deliver improved outcomes and access for patients, a thriving workforce, financially sustainable organisations and a shift of investment away from acute hospital care into primary and community care services.

The programme directly responds to three high profile national policy documents published by NHS England, the Fuller Stocktake Report published in May 2022, the Darzi report published in 2024, and the NHS Neighbourhood guidelines published in January 2025. Neighbourhoods will be expected to develop plans to deliver the policy statements set out within these documents.

- Fuller report Microsoft Word - FINAL 003 250522 - Fuller report[46].docx
- Darzi report Independent investigation of the NHS in England - GOV.UK
- NHS England guidelines NHS England » Neighbourhood health guidelines 2025/26

The programme aligns directly with the 10 Year Plan, and we expect any subsequent national guidance published to support the delivery of integrated neighbourhood care.

The programme is focusing on developing 3 new models of care which are driven by an enhanced understanding of population health needs with an emphasis on tackling health inequalities. The new models are:

- Community Urgent Care (incorporating the Neighbourhood Access Service)
- Complex Care and Long-Term Conditions
- Planned Care

Sitting alongside the three new models within this programme, is a fourth area of work, Proactive Care, being led by the Local Authority. Proactive care is thought to underpin the other three models in the programme and be embedded with the Voluntary, Community & Social Enterprise (VCSE) sector.

To achieve greater consistency of approach to neighbourhood health, there is a requirement to focus on standardising 6 core components of existing practice:

- population health management
- modern general practice
- standardising community health services
- neighbourhood multi-disciplinary teams (MDTs)
- integrated intermediate care
- urgent neighbourhood services.

Single nominated organisations will need to take a leading role in working collaboratively

and effectively with the following key programmes/ initiatives:

- mental health, learning disability & autism (MHLDA)
- children & young people (CYP)
- over 65s
- elective care
- urgent care
- health inequalities
- prevention

Neighbourhoods must have specific plans to support management of these groups.

At the heart of the programme is the ambition to protect 50% of GP time to support an enhanced model of complex care and long-term condition management. Presently it is estimated that GPs spend up to 90% of their time on reactive care, responding to the overwhelming demand for same day access services.

To achieve this ambition, neighbourhoods will implement additional services which are fully integrated into the wider community urgent care model and add extra capacity. Retaining the GP Practice as the front door to the NHS, GP Practices will be able to directly book appointments into the newly designed Neighbourhood Access Service creating capacity to offer an enhanced complex care and long-term condition service. This will facilitate the movement towards protecting 50% of GP time as described above. This Enhanced Specification (ES) supports the delivery of 22% additional capacity using the allocated funding, which should be utilised for reactive appointments. There is an expectation within this ES that a further 18% of additionality will be found by efficiencies such as working at scale and integrated pathways. Together, these two elements deliver 40% additionality to provide reactive appointments, releasing 40% of GP time to support the enhanced model of complex care and long-term condition management offers.

Commissioners expect delivery ‘at scale’, led by a single nominated organisation that will be responsible for holding and managing programme funding (‘held funds’) for all GP Practices in that Neighbourhood. To prevent a model that disaggregates activity in the newly designed Neighbourhood Access Service back to registered GP Practices, all appointments will be accessible to all registered patients and temporary residents from all GP Practices within the geographical footprint of that Neighbourhood. Activity includes provision of general medical services required for immediately necessary treatment during the newly designed Neighbourhood Access Service opening hours.

Activity delivered in the newly designed Neighbourhood Access Service (to support the community urgent care model) is in addition to the General Medical Services (GMS) contract services already being provided within GP Practices as set out in part 8 of the GMS contract (clause 8.1.8), or any existing Local Enhanced Service (LES) and Directed Enhanced Service (DES) contracts. This additionality will receive the appropriate programme funding and will be delivered in a different ‘at scale’ model i.e. not delivered out of host GP Practices but easily accessible for patients.

Neighbourhoods will be required to work collaboratively with their patients and local partners to develop detailed operational plans for the Neighbourhood Access Service. Sites will make best use of existing estate, working with local providers to both access these, and deliver care.

The programme will evolve over time, and it is expected that Neighbourhoods remain responsive and flexible to the needs of Commissioners and the population of Northamptonshire throughout the duration of the contract.

This ES does not state precisely how the programme will be delivered. However, the requirement is for a fully integrated service that delivers efficiency, improvement and resilience with specified outcomes and deliverables. It is expected within this ES that a patient accessing the Neighbourhood Access Service will experience the same care as if they attended an appointment in their registered practice.

As such, the Neighbourhood Access Service must be able to demonstrate working towards achieving full interoperability, agreed with the ICB. This assurance is required such that the patient and workforce can use the full capability of clinical systems. While full interoperability is being developed, each site must have robust standard operating procedures for managing cross-site working. This includes, but is not limited to, the ability to view the patient's full health record, make referrals and request diagnostics. This digital interoperability should facilitate direct access to these functions as it would if the clinician was working within their main practice and should not generate additional administrative tasks.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	**Preventing people from dying prematurely**	**Yes**
Domain 2	**Enhancing quality of life for people with long-term conditions**	**Yes**
Domain 3	**Helping people to recover from episodes of ill-health or following injury**	**Yes**
Domain 4	**Ensuring people have a positive experience of care**	**Yes**
Domain 5	**Treating and caring for people in safe environment and protecting them from avoidable harm**	**Yes**

2.2 Local defined outcomes

- Improved outcomes and access for patients through implementation of new models of care and integrated pathways
- A sustainable primary care (and community care from the broader New Models programme) sector with investment shifted to community-based services
- Effective triage that facilitates right care, right place, right time
- Reduced A&E attendances, outpatient activity, and non-elective admissions through embedding a model of care that is proactive and preventative instead of episodic and reactive
- Empowered patients via increased self-management where clinically appropriate
- Empowered patients taking control of their health and wellbeing through increased use of the NHS app
- A service that is co-produced over time through diverse voices and via ongoing patient engagement
- High quality, efficient service delivery in line with statutory and policy requirements and Commissioner requirements. Quality covers three specific domains:
  - Safety (we do no harm to patients, and the environment is safe and clean)
  - Effectiveness (improved clinical outcomes) and
  - Patient Experience (quality of care and the delivery of personalised care)
- A service which is safe, patient-centred and meets patients' needs
- Increased patient and carer satisfaction through improved access to appointments via the Neighbourhood Access Service, provision of proactive care and continuity
- Increased staff satisfaction, recruitment and retention

- Improved public perception of the local health system

A Benefits Realisation Programme will be developed and agreed with the innovator sites which will focus on maximising the value and impact of changes made by delivering the detailed service specifications. The aim of the programme will be to ensure planned benefits are achieved and sustained. It will take a structured approach to managing change, ensuring that resources are used effectively, and improvements are delivered as intended. This involves identifying, planning, tracking and evaluating benefits throughout the lifecycle of the programme.

Benefits realisation will focus on both financial and non-financial benefits. For example:

- Financial benefits
    - cashable (cost savings)
    - non-cashable (cost avoidance and productivity improvement)
- Non-financial benefits
    - effectiveness (service quality including patient outcome, safety, experience and satisfaction)
    - statutory or policy requirement
    - supporting strategic goals (including social, environmental and wider economic aims)
    - risk mitigation and avoidance
    - workforce (morale, retention and recruitment)
    - essential replacement
    - reputation

Innovator neighbourhoods are funded on the premise of system savings in excess of the investment, utilising baseline trajectories to measure impact.

The agreed benefits realisation programme and information schedule will be appended to this specification and updated with the programme board on an annual basis.

2.2.1 Monthly Reporting

As part of the benefits realisation programme, a suite of data will be required for monthly reporting. Some of this data will be captured internally by the ICB, and some will be required to be submitted monthly by the Single Nominated Organisation. To support the Single Nominated Organisation, each GP Practice must nominate a named reporting lead responsible for timely and accurate data submission.

The information schedule will contain a number of fixed areas including, but not limited to:

- Activity and performance (Neighbourhood Access Service)
    - utilisation rates (including practice breakdown)
    - outcomes
    - onward referrals
    - diagnostics
    - access times
    - repeat attendances
    - patients not receiving an appointment due to lack of availability
- Activity and performance (Complex and Long-Term Conditions Service)
    - utilisation rates
    - outcomes
    - onward referrals
    - diagnostics
    - access times

    - repeat attendances
    - appointment lengths
    - specific clinics for specified cohorts

- Quality
    - complaints
    - incidents
    - safeguarding
    - infection prevention control (IPC)
    - patient experience
    - effectiveness
    - safety
    - training/workforce

Narrative will be expected to explain performance variances, actions taken to address underperformance (if applicable), evidence of implementation of patient and staff feedback, and learning from incidents.

Additional measures will be added based upon the specific design of the model in the neighbourhood contracted.

Practices (and therefore neighbourhoods) who do not submit complete data within the agreed timeframe, neighbourhoods not meeting the activity requirements of this enhanced service, and neighbourhoods where the forecast outcomes are not met, will be subject to performance review by the programme board and any actions identified. Actions could include: further interrogation of data, remodelling capacity and activity, more frequent data submissions, quality reviews, and ultimately termination and or clawback of the contract.

3. Scope

This ES is offered by the Commissioner to GP Practices in selected Neighbourhoods within the geographical footprint of Northamptonshire ICB.

This ES is set out as follows:

- Part A - Neighbourhood Access Service
- Part B - Enhanced Complex Care and Long-term Condition Service

Part A of this ES is designed to cover and/or support aspects of clinical care **which are beyond** the scope of essential (part 8 clause 8.1.8.) or additional services (part 9 clause 9.1.4.) as set out in the GMS contract. Part B is designed to cover and/or support aspects of clinical care **which are included** in the essential or additional services. Part A (the Neighbourhood Access Service) is designed to shift activity around reactive care, enabling released time for clinicians to deliver Part B, aligning to the 50/50 ambition between reactive and proactive care.

No part of this ES by commission, omission, or implication defines or redefines essential or additional services.

GP Practices are offered the opportunity to sign up to this ES on a phased-roll out plan due to restricted funding. GP Practices will be allocated to phases based upon readiness to meet the requirements of this specification and expected impact in the neighbourhood. By signing up to deliver this ES, a GP Practice agrees to a variation of its general medical services contract limited solely to the provisions set out in this ES specification.

This variation does not extend to or imply automatic participation in any other national or local enhanced services, or future contractual changes unrelated to this ES.

This specification is referred to as this "ES". In this ES:

- "Commissioner" refers to the organisation with responsibility for setting the terms of this ES, which is the Integrated Care Board (ICB). The Commissioner also has responsibility for contract managing these ES arrangements
- "GP Practice" refers to a provider of essential general medical services to a registered list of Patients under a GMS contract, Personal Medical Services agreement or Alternative Provider Medical Services contract with Northamptonshire ICB who has agreed with the Commissioner to deliver this ES
- "Single nominated organisation" refers to the managing authority for 'held funds' to support programme delivery on behalf of all constituent GP Practices for that Neighbourhood for the duration of this ES. The single nominated organisation also has responsibility for programme governance
- "Innovator site" refers to a pilot site for the newly designed Neighbourhood Access Service
- "at scale delivery" refers to providing healthcare services across a larger geographical area or patient population than a single GP Practice or organisation can manage alone
- "additional capacity service" refers to activity delivered in the newly designed Neighbourhood Access Service (to support the Community Urgent Care model) which is additionality to the GMS contract services already being provided within GP Practices as set out in part 8 of the GMS contract (clause 8.1.8), and also additionally to DES and LES contracts

In this ES, words importing the singular include the plural and vice versa.

Governance

Commissioners expect each Neighbourhood to have a single nominated organisation which is compliant with the organisation requirements set out in appendix 1.

Each single nominated organisation participating in this ES will:

- ensure that GP Practices within the Neighbourhood are signed up to deliver collaboratively through that organisation for that Neighbourhood
- be the managing authority for Part A 'held funds' under the programme on behalf of all constituent GP Practices for the Neighbourhood for the duration of this ES
- establish a governance structure including a Programme Board with representation from each GP Practice within the Neighbourhood to provide high-level oversight and governance for the programme. For the avoidance of doubt, the ICB relationship will be with the Board and not with the individual GP Practices within the Neighbourhood. The Board will ensure the programme aligns with strategic goals, allocates resources, manages risk and makes decisions
    - the Board will co-ordinate and supply an annual neighbourhood plan for achieving improved outcomes for patients with complex care and long-term conditions
    - the Board will co-ordinate the approach to receiving of funds associated with the contract
- generate and participate in a Benefits Realisation Programme to ensure the programme delivers the anticipated value, improves services and contributes to strategic goal.
- in partnership with Commissioners, undertake the work required to fully understand if activity and financial assumptions have been modelled correctly. This will require an "Open Book" approach to sharing information and showing

activity, performance, financial and workforce breakdowns, plus any other information that is critical to the future success of commissioning this service. Financial information required will include a detailed spend breakdown. This will help to shape the long-term commissioning of services through shared learning from Innovator sites. Any changes to the assumptions will be updated in the ES as part of the annual contracting mechanism

- in addition to the usual management of programme risks and issues, immediately inform the ICB of any major risks and issues that occur which have the potential to de-rail the programme e.g. 50% of GP Practices failing to utilise the Neighbourhood Access Service activity against key performance indicators. In such cases, the ICB will write to all GP Practices to take immediate remedial action
- ensure GP Practices and/or Single Nominated Organisations respond to freedom of information requests within 20 working days in line with the Freedom of Information Act 2000 regulations
- have policies in place with agreed roles and responsibilities with partner practices in the neighbourhood
- publish quarterly financial and activity reports at the practice level, including:
    - appointment contribution and utilisation
    - financial allocation and spend
    - patient feedback themes
    - this data will be shared with all practices and the ICB to ensure transparency and equity

Terms

This ES begins on 1 April 2026 and shall continue until it is terminated, with a minimum term of two years, in accordance with the following:

- either party may terminate this contract without cause and at any time upon giving 6 months written notice to the other party
    - notice may be given if the outcomes outlined above are not met
- such termination will be effective on the date stated in the notice
- if a practice chooses to give notice but the other practices in the neighbourhood wish to continue this ES, the viability of the neighbourhood in continuing to deliver this ES will be at the discretion of the commissioner

In the circumstance that a GP Practice serves notice or informs the commissioner of their intention to merge with another partnership, the Commissioner will assess the impact on the Neighbourhood Access Service, including its future viability and has the right to terminate this contract if it feels the minimum standards cannot be met as a result.

The contract and term will be subject to ongoing collaborative review and refinement.

GP Practices in the designated neighbourhood that choose not to participate at the commencement of the contract may submit a written request to participate to the ICB endorsed by the programme board. The ICB will consider this request in line with available funding and in line with the annual planning decision making process.

Payment under this ES is conditional on all participating GP Practices in the designated neighbourhood:

- entering into this ES, including any variations and updates to this ES
- complying with the requirements of this ES
- aligning with and automatically participating in any variation to the GMS contract

Each GP Practice participating in this ES alongside the single nominated organisation will:

- have a neighbourhood MOU between each practice and the single nominated organisation. The commissioner will provide an optional template, however practices and/or neighbourhoods are able to use their own template with commissioner approval
- inform the commissioner of any proposed changes to any sub-contractors, for approval
- comply with any reasonable request for information from the Commissioner relating to the provision of the services pursuant to this ES, including monthly data submissions
- have regard to all relevant guidance published by the Commissioner or referenced within this ES
- provide information to patients about the services that are available as a result of participation in this ES
- ensure that it has in place suitable arrangements to enable the lawful sharing of data, including patient records, to support the delivery of the services, business administration and analysis activities under this ES in line with data protection legislation; and
- ensure that any sub-contracting arrangements related to the provision of services under the ES, comply with the requirements set out in the statutory regulations or directions that underpin its general medical services contracts in relation to sub-contracting, which will also apply to any arrangements to sub-contract services under the ES
- ensure suitable translation and interpretation services are available to ensure effective communication between patient and clinician
- facilitate reasonable adjustments to support patients in access, attendance and care delivery
- effectively utilise the service as per patient demand, aiming for full utilisation of the neighbourhood access model across the neighbourhood.
- ensure that safeguarding responsibilities, policies and procedures are in place and adhered to
- GP Practices will retain responsibility for booking and cancellation management for their registered list and temporary residents
- the responsibility sits with the LES holder and the single nominated organisation to ensure they have the appropriate insurances (for example, medical and public liability) in place to support the delivery of the services.
- continued access to Part A ‘held funds’ is conditional on demonstrable delivery of Part B commitments. Practices failing to meet minimum thresholds for proactive care delivery may be subject to:
    - Performance improvement plans
    - Reallocation of appointments in the Neighbourhood Access Service
- adhere to the legislation underpinning GMS and PMS arrangements. This ES provides additional capacity based upon GMS activity, and as such, requirements around safeguarding, incident response, staff accreditation and training apply as per the GMS contract (this list is not exhaustive)
- have business continuity processes in-place to ensure service sustainability as a result of unexpected or emerging interruptions to workforce, estate, IT or other events
- attend the workshops held in the design phase to work through clinical systems interoperability and patient stratification management, with a developed or developing local responsibility policy or standard operating procedure

Exceptions:

- GP Practices cannot opt out of handing over the Part A programme funding ('held funds') to the single nominated organisation. The single nominated organisation is responsible for the financial management of the programme within the Neighbourhood
- the single nominated organisation must not disaggregate the 'held funds' back down to GP Practices for GMS, DES or LES contract services already being provided within GP Practices
    - The 'held funds' must only be used for additional capacity (above the existing capacity baseline) for the Neighbourhood Access Service as set out in Section 3 Scope of this ES. It must not be used for services already funded or for the running of GP Practices, for example, to pay for a new telephony system as access to the newly designed Neighbourhood Access Service will be via the usual front door of the GP Practice
    - For the avoidance of doubt, DES & LES appointments are not included as part of the additional activity, but we would actively encourage GP Practices to review opportunities to work together to enhance the Neighbourhood Access Service utilising existing funding streams. Examples of this could include a neighbourhood vaccination service, medicines optimisation, First Contact Physiotherapy via the ARRS funded scheme
    - Restricting access to some GP Practices is not permitted and all service appointments must always be available to all patients within the Neighbourhood during the newly designed Neighbourhood Access Service opening hours. It will be for the programme board to monitor and ensure equity of utilisation of appointments by each GP Practice, and the ICB will require visibility of this data
- utilising the additional Neighbourhood Access Service capacity for patients with complex health needs and long-term conditions is not routinely advised as continuity remains paramount for this cohort. An exception to this is where the patient requires specialist input - explained further in the innovation section
- wrap around services such as practice nursing, General Practice Administration Services, ARRS funded staff, whilst can be fully integrated into the model design are not part of the designated additional activity and are therefore not funded as part of this ES

<u>Aims and Objectives of Service</u>

The aims of this ES are to:

- support GP Practices and the single nominated organisation to work collaboratively with patients and Neighbourhood partners to design and implement an 'at scale' operating model which is GP-led, high quality, patient centred, community-based offer via the newly designed Neighbourhood Access Service. The service will provide additional appointments primarily for patients who present with conditions where recovery is generally expected. The scope of practice must include illness in adults and children of any age.
- provide funding ('held funds') for additional GP, ANP/ ACP diagnostic appointments as part of a Neighbourhood Access Service, which is expected to deliver 15.2 appointments per week, per 1,000 patients registered in the neighbourhood, with surge capacity for 13 weeks over the winter period at 18.2 appointments per week, per 1,000 patients registered with the neighbourhood.

- provide Health Care Assistant support to GPs and ANP/ACPs to ensure effective delivery of services (these appointments do not count as activity delivered under this contract).
- support development of protected time freed up through the newly designed Neighbourhood Access Service, to implement more proactive care for those with complex needs and/ or long-term conditions, providing continuity and less reactive care. This ambition is planned to improve outcomes for this patient cohort, including clinical outcomes and patient opinions of care.
- to provide more proactive, personalised care with support from a multidisciplinary team of professionals to people with more complex needs, including, but not limited to, those with multiple long-term conditions
- work collaboratively and effectively with neighbourhood teams and wider system partners to provide more intensive support to patients. This should include MHLDA, CYP, over 65s, Elective Care, Emergency Care, Health Inequalities, and Prevention to support people who are unwell to be cared for safely at home, and for those requiring hospital treatment, to ensure safe and effective transfers into and back from hospital.
- ensure that service plans deliver the policy statements as set out in the documents listed in section 3 (Scope) of this ES.
- ensure that activity delivered within the newly designed Neighbourhood Access Service is additionality to the GMS contract services already being provided within GP Practices as set out in part 8, clause 8.1.8. and part 9, clause 9.1.4. of the GMS contract. Additionality must be offered as a discrete service that will be easily and equally accessible for all patients within the Neighbourhood for reactive appointments.
- work with wider system partners to fully integrate the newly designed Neighbourhood Access Service into the wider community urgent care and planned care models. The programme board will take a lead role working alongside partners to align and integrate other services to the neighbourhood access model.
- work with wider system partners on developing pathways which incorporate multi-modal access options including online, telephone and face-to-face and to empower patients to actively provide and update their GP Practice with this information online.
- ensure that risk stratification and effective triage is in place to ensure the appropriate patient presentation in the Neighbourhood Access Service and alignment with the enhanced complex care and long-term condition management element of the specification (see below for an example taken from the Fuller Report on how patients have been risk stratified to support triage to a same day access service. Locally, patients are stratified into 11 Patient Needs Groups (PNG) which will be used to triage the patients at a neighbourhood level, and further stratification can be used locally where considered appropriate (one example below).

Example Model

The Foundry Health Centre is a single practice PCN in Sussex with 28,500 patients. Since 2019, it has sought to improve access and keep patients out of hospital. Patients are streamed using systematic triage and clinical judgement and identified as green (generally well – continuity less important), amber (long-term conditions – continuity

important; appropriate reactive care delivered), and red (vulnerable or complex – continuity paramount; proactive care given).

- to realign existing workforce to support the most vulnerable and complex patients to stay at home and access care in the community. This will, over time, contribute significantly to efforts to reduce growth in hospital demand and signal a shift away from a hospital-centric model of care that is no longer suited to the population we serve.
- to develop an annual neighbourhood plan for achieving improved outcomes for patients with complex care and long-term conditions. Delivery of this will be key to the success of the Innovator sites and will drive the benefits realisation needed for the programme to be extended to other neighbourhoods and / or to be sustained longer term.
- ensure robust financial management, programme governance, programme evaluation and benefits realisation through the single nominated organisation to support programme evolution.

Workforce

To support increased participation and retention in Primary Care, GP Practices will need to attract new staff into the service and create the backdrop that allows their roles to be redesigned and attractive.

Clinical resource that is being used from GP Practices to support the Neighbourhood Access Service must be backfilled to protect capacity for complex care and long-term condition management.

The Neighbourhood Access Service will need to develop and agree an approach to training nurses, medical students, foundation year and GP registrars. The single nominated organisation will:

- work with local universities and the Northamptonshire Primary Care Training Hub to offer structured placements
- ensure students are supervised by appropriately trained clinicians
- provide opportunities for interprofessional learning
- include student feedback in service improvement cycles

Part A. Neighbourhood Access Service

Service Description

The Neighbourhood Access Service is a neighbourhood same day or reactive access service which is fully integrated into a wider community urgent care model. Retaining the GP practice as the front door to the NHS, practices would be able to directly book appointments into the service creating capacity to deliver an enhanced complex care and long-term condition service. This will be in addition to existing primary care services, such as core GMS commissioned services and Pharmacy First.

This service is to be provided through an at-scale model, with the level of appointments required matching with the financial and activity modelling. The appointments may be provided from a central location or split across the neighbourhood, depending on the specific neighbourhood design. However, all appointments under this service will be available to all patients registered with a practice within the neighbourhood.

In most cases, the Neighbourhood Access Service should be designed to support patients with low complexity presentations requiring less continuity of care. This work will be supported by patient stratification into patient needs groups (PNG) as set out above.

The additional appointments available within the Neighbourhood Access Service should also be used to support ambulance service colleagues, where an appointment, prescribing or clinical support in General Practice is more appropriate than a conveyance to hospital for patients that would ordinarily meet the criteria for the Neighbourhood Access Service, e.g. where continuity is not paramount. In this case, a conversation between East Midlands Ambulance Service (EMAS) and the patient's registered practice would be expected to ensure appropriateness.

Consideration of improving health inequalities must be a fundamental to the design of the neighbourhood operating model, and inequalities should continue to be addressed by this service in addition to any other contracts or enhanced services. This includes identification and definition of:

- groups most at risk
- Core20PLUS5 cohorts

The operational model should clearly state the intent to:

- reduce health inequalities across specific groups
- improve access, outcomes and experiences for disadvantaged populations
- embed equity into service design, delivery and evaluation

Activity Modelling and Expectations for Delivery

The funding attached to this ES is expected to all be spent on the Neighbourhood Access Service (Part A) and innovation, and not the Complex Care and Long-Term Conditions Service (Part B).

The funding attached to the ES is expected to provide 22% additionality for reactive appointments, with a further 18% to be found from the innovation section. Together, these are modelled to make up the 40% additional capacity required to achieve the ambitions of the programme which will enable the 50/50 model between reactive and proactive care. To achieve this, any capacity released as a result of this additionality, will deliver Part B of this ES.

The Neighbourhood Access Service is expected to deliver 15.2 appointments per week, per 1,000 patients registered in the neighbourhood. Surge capacity has been modelled and funded for 13 weeks over the winter period, where the Neighbourhood Access Service will be expected to provide 18.2 appointments per week, per 1,000 patients registered with the neighbourhood.

Modelling is based upon:

- 50% of appointments being provided by a GP/GP with an extended role (GPwER)/GP with specialist interest (GPwSI)/Consultant, and 50% by an Advanced Nurse Practitioner (ANP)/Advanced Clinical Practitioner (ACP)/ Allied Health Professional (AHP)
- appointments having a length of 15 minutes, with 12 appointments in a session.
- up to half of the appointments may be delivered virtually (online or telephone)
- ANP/ACP/AHP's delivering 24 appointments per day (whole time equivalent (WTE))
- GP/GPwER/GPwSI/Consultant's delivering 15 appointments per session

It is expected that innovator sites will test this model and provide evidence to support any proposed learnings which deviate from this.

Referral Processes

The single nominated organisation is to ensure newly designed Neighbourhood Access Service referrals are appropriately targeted and of good quality.

Where the patient cannot be diagnosed or treated by the newly designed Neighbourhood Access Service, they will be referred directly to alternative care services where appropriate. This should have documented agreements, for example, standard operating procedures and service level agreements. Such referrals should follow routine recording processes within the practice.

Referrals should be made directly from clinicians working within the Neighbourhood Access Service and should not be passed back to host practices to administrate. This includes any follow up services- referrals to specialist care, further diagnostic appointments, phlebotomy…

Pathology
Pathology requests should be made directly from clinicians working within the Neighbourhood Access Service and should not be passed back to host practices to administrate. This includes any follow up services- referrals to specialist care, further diagnostic appointments, phlebotomy… It is expected that any requested tests from the Neighbourhood Access Service will be reviewed in line with national requirements within a standard operating procedure agreed for the neighbourhood.

Medicines Optimisation
The prescribing budget for the Neighbourhood Access Service will form part of the primary care prescribing budget and will be calculated for 2026/27 and onwards with regular monitoring, oversight and review of financial assumptions:

- the service will prescribe according to local guidance for prescribing and medicines optimisation
- the service will follow the local formulary and traffic light classifications when prescribing
- the service will follow local and national recommendations to ensure value from medicines use

All healthcare professionals must work within their professional competence when prescribing and administering medicines and when providing medicines advice. GP practices and the single nominated organisation will support individuals to ensure they are appropriately qualified and skilled for any prescribing role undertaken.

Consideration of the Prescribing Achievement Framework and the neighbourhood approach for achieving and monitoring of this will be discussed during the design phase of the service with the neighbourhood and the ICB.

Exceptions

- patients with complex health needs and long-term conditions should routinely be seen in their own GP Practice under normal general practice arrangements, and not in the newly designed Neighbourhood Access Service - see Innovation section below.
- home visiting is not covered within this specification and will not be delivered as part of the Neighbourhood Access Service.

Population Covered
The Neighbourhood Access Service will be available to all patients registered with the partaking practices in the neighbourhood, including temporary residents. This covers all ages.

Neighbourhoods must develop and implement a protocol for the identification of patients for the Neighbourhood Access Service, this should include:

- ensuring the patient is informed that they will be seen in the Neighbourhood Access Service and that they understand what this means (including that it may not be attending an appointment in their own GP Practice). Shared decision-making principles should apply.
- ensuring that patient triage is in place to ensure appropriateness, quality control and relevant clinical presentation.

The Neighbourhood Access Service will be available for children under 5 where clinically appropriate, and appointments for patients aged under 5 will be provided face-to-face.

Opening Hours

The Neighbourhood Access Service will be operational 252 days per year (weekdays excluding bank holidays), opening hours will be a minimum of those in the core GMS contract. Neighbourhoods have the option to extend the opening hours above core hours, and neighbourhoods may wish to align the Neighbourhood Access Service with their Enhanced Access services, however activity provided under Enhanced Access services will not be counted as activity within this enhanced service. However, the single nominated organisation may be required to extend opening hours should the Commissioner request it.

Delivery of all care related to this Neighbourhood Access Service will be within the geographic footprint of Northamptonshire Integrated Care System (ICS), and the appointments available through this service should be within the geographic footprint of the neighbourhood. Premises will be accessible by public transport and have car parking facilities.

GP Practices and the single nominated organisation will manage occupation of any premises in such a way as to provide the best possible experience for patients arriving, leaving and using the facility, and to support the delivery of the Neighbourhood Access Service.

GP Practices and the single nominated organisation will be responsible for ensuring the locations used offer a safe environment. This includes ensuring the premises continue to meet regulatory standards and any applicable NHS standards; remain suitable for delivery of the service; and be safe and appropriate for users with particular needs. A contingency plan will be in place which includes plans to address operational contingencies which may occur.

Part B. Complex Care and Long-Term Conditions Service

Service Description

The Complex Care and Long-Term Conditions service is a proactive and preventative service, provided in the time released by the additional capacity as set out in Part A. Retaining the GP practice as the front door to the NHS, practices will proactively engage with patients in higher PNG groups to provide higher levels of continuity of care, with the option for longer appointments, check-ins and more detailed consultations. Care under this element of the specification should be personalised and consider elements of prevention and access to community services to ensure patients understand the holistic offer of support.

The service will be provided through registered GP Practices, with the level of appointments required matching with the financial and activity modelling.

In most cases, the Complex Care and Long-Term Conditions service should be designed to support patients with high complexity, requiring more continuity of care. This work will be supported by the patient stratification into PNG groups as set out in the Terms section above. Using the patient stratification PNG groups, patients will be triaged and highlighted for continuity of care where appropriate, allowing them to be booked into this capacity. This element of the service is also all age.

The appointments available within the Complex Care and Long-Term Conditions Service should also be available for ambulance service colleagues to refer, where an appointment in General Practice is more appropriate than a conveyance to hospital for patients that would ordinarily meet the criteria for the Complex Care and Long-Term Conditions Service, e.g. where continuity is paramount.

Continuity

To provide continuity of care for patients with complex needs and/ or long-term conditions. ICB priorities as indicated must be prioritised; the list below suggests areas of focus but is not exhaustive:

- chronic obstructive pulmonary disease (COPD)/ Asthma (ICB priority) - management plans including children and provision of standby medication; post exacerbation and post admission reviews.
- cancer - opportunity for GP to check in with patients via telephone once patient consent has been gained.
- diabetes - support and management plan for newly diagnosed patients, advising and referring to surveillance and education programmes, and/or, manage and optimise poorly controlled diabetic patients with appropriate signposting and advice for lifestyle support and weight management if required.
- frailty including dementia - including support for family and carers. RESPECT plans (see below).
- low level anxiety and depression - to reduce escalations and support provision of personal care.
- chronic disease/learning disability (LD) reviews/serious Mental Health check-ups - opportunity for GPs to support complex and higher-level needs with potential for Nurse Practitioners to book into these from chronic health reviews.
- end of life (EoL) - opportunity for patient to have a named GP to support home visits and check in via telephone with the patient, family and carer once consent has been gained. RESPECT plans (see below).
- RESPECT plans (ICB priority) - not limited to EoL patients and provides the opportunity to confirm or initiate RESPECT conversation and document on NCR when patients can voice their wishes in case of an emergency/ EoL/LD supported discussion. Discussions could be completed over more than one appointment although the form must be completed all at once. Consideration should be made for patients appropriate to have a conversation around RESPECT plans, and this should specifically be considered wider than the EoL cohort.
- high intensity users - to reduce escalations and daily telephone contact from patients, and support identification through high usage flag.
- housebound patients - to support GP home visits which can be ad hoc e.g. different paramedic, nurse
- polypharmacy - specific support around patients around managing polypharmacy, in addition to medication reviews carried out routinely.
- complex pain - specific support for patients with complex pain to provide continuity of care and access where required.
- unexplained medical symptoms - continuity of care for patients experiencing unexplained medical symptoms, particularly around those known to be high users

of services, but also for those who may have exacerbations and not use services where needed.
- patients with delayed discharges or significant length of stay where it is clinically appropriate to support them in the community.

In addition:
- named GP - for some of the patient cohorts above, for example, COPD, frequent admissions, Frailty including Dementia providing, continuity of clinician to support discussions. Ensure parity of physical and mental health.
- complex CYP cohorts, for example mental health (MH), attention deficit/hyperactivity disorder (ADHD)/ autism spectrum disorder (ASD), and eating disorders, may best be supported where GPs are upskilled/ have GPwSI/ GPwER roles with support from a Specialist. Care pathways should be developed with reference to national guidance and secondary care involvement where appropriate.
- further work towards the Core20PLUS5 agenda in the local population.
- Readmissions/delayed length of stay - support in discharging patients whereby increased support in the community can facilitate a safe discharge and prevent delayed length of stay. It is anticipated that impact of the programme on this area will increase alongside the phasing in of neighbourhoods.

Appointments
Proactive care appointments can be longer and of variable length dependant on the patient cohort, with consideration of the Make Every Contact Count programme. Longer appointments for this cohort will be specified and agreed (across the neighbourhood and with the ICB) within the individual neighbourhood design. This will enable GPs to have a fuller discussion with patients and develop care plans specific to patient needs. The additional time allowed provides the opportunity to consider and discuss onward referral to 'outside the box' support schemes such as Breathing Space, Social Prescribing and Ageing Well.

Each neighbourhood must deliver a minimum number of proactive care appointments per 1,000 registered patients per month, aligned with the capacity released through Part A of this specification. However, there is an understanding that longer appointments for this element of the specification will lower the overall number of appointments delivered (while maintaining the number of sessions and as such the clinical time). This is expected to be split across the general practices within the neighbourhood, and these appointments must be:
- Clearly coded in the clinical system
- Allocated to patients in PNG groups 8–11 (or local equivalents)
- Tracked for continuity (e.g. same clinician, follow-up intervals)
- Management of proactive care rotas on GP Clinical systems will be key to protecting proactive care appointments. These slots <u>must not</u> be utilised for reactive care.

Referral Processes
Referrals will follow the standard procedures of general practice, as these appointments will be delivered within the registered practice.

Discharge Processes
Patients with delayed discharges, or significant lengths of stay, where it is clinically appropriate to be supported in the community, should be supported through this element of the specification. This may entail a limited series of appointments to ensure the

patient is recovering in the community as expected, or a more detailed discussion around their holistic care, for example.

Exceptions
GP Practices will not receive additional funding for protected time freed up for complex care and long-term condition management under this ES as payment is already covered under the GM contract and DES/LES contracts.

Population Covered
Neighbourhoods must develop and implement a protocol for the identification of patients for complex care/ long-term condition management reviews, this should include:

- ensuring the patient is informed and that they understand what this means. Shared decision-making principles should apply.
- ensuring that patient triage is GP led; where it is not initiated by a GP, referrals must be reviewed by a GP ahead of the referral being submitted to ensure appropriateness, quality control and relevant content.
- ensure that risk stratification and effective triage is in place to ensure the appropriate patient presentation in the Neighbourhood Access Service and alignment with the enhanced complex care and long-term condition management element of the specification (see below for an example taken from the Fuller Report on how patients have been risk stratified to support triage to a same day access service. Locally, patients are stratified into 11 Patient Needs Groups (PNG) which will be used to triage the patients at a neighbourhood level.

Innovation

Where new technologies and innovation are not currently available it is expected that the single nominated organisation would move towards new ways of working during the lifetime of the ES in negotiation and consultation with Commissioners.

Commissioners are particularly keen to support any areas of innovation, particularly for patients (regardless of complexity / PNG group) requiring specialist input from a GP with Specialist Interest/ GP with Extended Role (GPwSI/ GPwER) or Consultant. The Royal College of General Practitioners (RCGP)'s position on extended role accreditation, and reaccreditation, is outlined in the RCGP guide to clinical extended roles General Practitioners with Extended Roles. The RCGP does not accredit GPs with extended roles but currently supports the British Association of Dermatologists to provide an accreditation programme for dermatology GPwERs. GPwSI/ GPwER activity can be counted as activity within the Neighbourhood Access Service (see Activity section). Where neighbourhoods are looking to engage GPs with Extended Roles or Consultants through this service, they will need to work with Northamptonshire ICB to ensure we are meeting all requirements.

This specification was written after receiving the 10 Year Plan guidance, however prior to any specific delivery guidance. As Neighbourhood Health Centres, Family Hubs and other neighbourhood initiatives develop, neighbourhoods delivering the New Models Programme should consider aligning services with these initiatives to support the Making Every Contact Count agenda and increased patient accessibility.

Innovator Sites

Innovator sites will receive Innovator status and as such will be required to share learning to support other Neighbourhoods post phase 1 as follows:

- test the model of care design to establish suitability for future Innovator sites and shared learning.

- test new ways of working for ‘at scale’ delivery, for example,
    - patient triage for presentation at service
    - referral process
    - scaling of functions such as pathology
    - support for a pooled budget approach for medicines management
    - reviewing activity and financial modelling assumptions to help to shape the long-term commissioning of services through shared learning
- share learning with the ICB and other Neighbourhoods in support of future Innovator sites and programme evolution.
- support the ICB with presentations and updates at local, regional and national level, including but not limited to, virtual and face to face meetings, training and education events, and conferences in support of wider shared learning.
- have specific plans to support management of population groups as set out in above.
- be required to work with Commissioners to fully understand if the activity and financial assumptions have been modelled correctly. This will require an Open Book approach to sharing information and showing activity, performance, financial and workforce breakdowns, plus any other information that is critical to the future success of commissioning this service. Financial information required will include a detailed spend breakdown. This will help to shape the long-term commissioning of services through shared learning from Innovator sites. Any changes to the assumptions will be updated in the ES as part of the annual contracting mechanism.

Service Improvement

GP Practices and the single nominated organisation to continually review the Neighbourhood Access Service and develop and adhere to priorities in line with national and local guidance.

GP Practices must offer each patient the opportunity to feedback on the service experience; any questionnaire or method of engagement must include the entire patient pathway and seek views on making an appointment, the environment of where the diagnostic took place as well as their view on the diagnostic procedure. The single nominated organisation must collate all patient experience and demonstrate where improvements and change have been made because of patient feedback.

The single nominated organisation must engage with all GP Practices covered in the contract. Feedback to be sought on administration processes, quality of reports, and directive clinical advice given within the report to be gained. Again, the single nominated organisation must collate all data and demonstrate where changes and improvements have been made because of clinical feedback.

Pricing Structure

The ‘actual’ list size of the GP practice as of 1 January will determine the contract value for the following financial year. For example, the list size of the GP Practice on the 1 January 2026 will determine the contract value for the period 1 April 2026 - 31 March 2027.

The annual contract value is derived from the 1 January list size multiplied by £26.33. The annual contract value will be uplifted by Review Body on Doctors' and Dentists' Remuneration (DDRB) from 2027/28 onwards.

Interdependence with other Services/Providers

The service will be required to work collaboratively and effectively with multiple services and providers, the following list is not exhaustive:

- end of life
- mental health learning disability and autism
- children and young people
- over 65s
- elective care
- emergency care
- health inequalities, and prevention
- specific winter surge schemes
- East Midlands Ambulance Service
- Northamptonshire Healthcare NHS Foundation Trust
- University Hospitals of Northamptonshire

The neighbourhood practices and the Single Nominated Organisation is expected to work with all system partners to ensure integration with care pathways.

4. Applicable Service Standards

4.1 Applicable national standards (eg National Institute for Health and Care Excellence (NICE))

Where not specifically stated within this specification, the provider will follow the guidance outlined in the GMS contract.

The service will be developed in the context of current national and local best practice and including:

- Health and Care Act 2022
- Children and Families Act 2014
- Children Act 1989
- Children Act 2004
- Human Rights Act 1998
- NHS Act 2006
- Equality Act 2010
- Neighbourhood Health Guidelines 2025/26
- Fuller Stocktake Report 2022
- Independent Investigation of the NHS in England (Lord Darzi) 2024
- Patient Safety Incident Response Framework 2024

The above list is not exhaustive, and other relevant guidance or standards should be adhered to. NICE guidance should be adhered to for all related decisions.

4.2 Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

- RCGP Guide to GP Clinical Extended Roles 2021

4.3 Applicable local standards

- North Northamptonshire Special Educational Needs and Disabilities (SEND) and Inclusion strategy (2024)
- West Northamptonshire SEND and Alternative Provision Strategy (2023)
- Children and Young People Joint Strategic Needs Assessment (JSNA), Northamptonshire County Council (2024)

- Northamptonshire Safeguarding Adult Board and Safeguarding Children's Partnership Policies and Procedures
- To implement a patient safety framework in line with NHS Primary Care Patient Safety Strategy 2025 that includes the sign up to Learn from Patient Safety Events (LFPSE) requirements in sharing information with ICB
- Service user data is kept confidential, with adherence to the Caldicott principles (2020) and DPA 2018/UK GDPR
- Providers must be registered with the Information Commissioners Office (ICO).
- Providers must have completed a Data Protection and Security Toolkit (DPST).
- Have regard to the principles in the Code of Practice on Confidentiality and Disclosure of Information
- Must comply with relevant health and safety regulations that apply to all NHS providers

The above list is not exhaustive, and other relevant guidance or standards should be adhered to. NICE guidance should be adhered to for all related decisions.

4.4 Equality and Inclusion requirements

The ICB is committed to preventing discrimination, valuing diversity, and achieving equity of opportunity. The availability of care and quality of service must be based on individual clinical need and should be equally available to all regardless of:

- gender
- age
- race
- religion
- sexual orientation
- disability
- marital status
- gender reassignment
- pregnancy and maternity

Keeping the above list of protected characteristics in mind, consideration should be given to:

- the privacy and dignity of patients by ensuring informed consent for treatment and access to a chaperone is available
- times and location of service provision
- access to patient transport and interpretation services

5. Location of Provider Premises

The Provider's Premises are located at:

Appendix 1

Due Diligence It is a requirement of this Enhanced Service that any Single Nominated Organisation meets the criteria outlined below.	
1	Please provide the registered company name and address of the organisation who will act as the Single Nominated Organisation.
2	Please indicate the trading status of the Single Nominated Organisation (i.e. NHS Organisation (please specify), a public limited company, a private limited company, a limited liability partnership, other partnership, sole trader, other (please specify).
3	Please indicate whether any of the following classifications apply to the Single Nominated Organisation: ○ Voluntary Community and Social Enterprise (VCSE), ○ Small or Medium Enterprise (SME), ○ sheltered workshop, ○ public service mutual, ○ none of the above See UK definition of Small or Medium Enterprise (SME): https://www.gov.uk/government/publications/fcdo-small-to-medium-sized-enterprise-sme-action-plan/small-to-medium-sized-enterprise-sme-action-plan
4a	Please indicate whether the Single Nominated Organisation is: a) a Prime Contractor and will deliver 100% of the key contract deliverables yourself; b) a Prime Contractor who will use third parties to deliver some of the services; c) a Prime Contractor but will operate as a Managing Agent and will use third parties to deliver all of the Services; d) a Consortium but not proposing to create a new legal entity; or e) a Consortium and intend to create a new legal entity/Special Purpose Vehicle (SPV).
4b	If you selected option b) or c), please provide details of your proposed model that includes members of the supply chain, the percentage of work being delivered by each sub-contractor and the key contract deliverables each sub-contractor will be responsible for. Please state "Not Applicable" if this does not apply to the Single Nominated Organisation.
4c	If you selected option d), please provide details of your consortium to explain the alternative arrangements i.e. why a new legal entity is not being created. Please note that the Commissioner may require the consortium to assume a specific legal form to support a contract, to the extent that it is necessary for the satisfactory performance of the contract. Please state "Not Applicable" if this does not apply to the Single Nominated Organisation
4d	If you selected option e), please provide details of your consortium, current lead member and intended SPV/new legal entity. Please also provide details of which member(s) will be underwriting the SPV/new legal entity and any existing agreements between the parties in relation to your proposed structure. Please state "Not Applicable" if this does not apply to the Single Nominated Organisation.

5a	Please confirm whether you (or any consortium member or material sub-contractor named above) are on the debarment list. (https://assets.publishing.service.gov.uk/media/67c19349750837d7604dbcd6/Debarment_List_revised.pdf)
5b	If you have responded "Yes" to 5a, as the inclusion on the debarment list is by virtue of a ground for discretionary exclusion (as defined in schedule 7 of the Procurement Act 2023 https://www.legislation.gov.uk/ukpga/2023/54/schedule/7), please provide relevant information and evidence to assure the Commissioner that the circumstances giving rise to the application of an exclusion ground(s) are not continuing or unlikely to occur again.
6a	Are you (or any consortium member or material sub-contractor named above) a relevant commercial organisation as defined by section 54 ("Transparency in supply chains etc.") of the Modern Slavery Act 2015 ("the Act")?
6b	If you have answered 'Yes' to 6a, are you (and all consortium members and material sub-contractors named above) compliant with the annual reporting requirements contained within Section 54 of the Act 2015? If 'Yes', please provide the relevant URL for your organisation (and all consortium members and material sub-contractors named above). If 'No', please provide an explanation as to why.
7	Please confirm that you and any consortium members or sub-contractors have the following policies: • Complaints Policy • Serious Incidents Policy • Clinical Governance Policy (e.g. the prevention of healthcare associated infections) • Safeguarding Policy (adult/children's) • Health and Safety Policy that complies with current legislative requirements, is regularly reviewed and contains appropriate risk assessments.
8	Has your organisation, (or any consortium member or sub-contractor named above) within the last five years, incurred enforcement notices from the Care Quality Commission or its predecessors, in relation to this or a similar service, served on any establishment run by your organisation? If yes, please provide details of the enforcement notice(s) and the action you (or any consortium member or sub-contractor named above) have taken to remedy the situation and explain why you believe this would not compromise your ability to deliver this contract. If none, please state "None" as your response.
9	Note: Providers of health care services in England are required to hold a Provider Licence unless they are exempt. Information regarding this is available at web link: https://www.england.nhs.uk/licensing-and-oversight-of-independent-providers/licensing/#background Do you (and all consortium members and material sub-contractors named above) hold a current NHS provider licence? If "Yes" please attach a copy of your (and all consortium members and material sub-contractors named above) licence. If "No" please provide an explanation of why you (or consortium members and material sub-contractors named above) do not require a licence or how you (and all

	consortium members and material sub-contractors named above) will you ensure this is in place for the start of this service?
10	Please state whether any clinical staff currently employed, sub-contracted, or otherwise engaged by the Single Nominated Organisation, have during the last 3 years, had their Professional Registration removed or suspended or whether they are currently under investigation. Where applicable, please provide relevant details of the removal(s), suspension(s) and/or investigation(s) and the action taken to ensure the ability to deliver this contract safely is not compromised. If none, state "None" as your response.
11	Please state your CQC registered number(s) and the organisation they relate to including the CQC registered number(s) for any consortium member and material sub-contractors named above), or, if you (or consortium members and material sub-contractors) do not already have this, please detail how you (and consortium members and material sub-contractors) will achieve registration before the service is due to commence. Alternatively, if you believe that CQC registration is not required, please explain why.
12a	Please provide a copy of your (the Single Nominated Organisation) audited accounts for the last two years. This should include a statement of the turnover, profit & loss account, current liabilities and assets, and cash flow for the most two recent years of trading for this organisation. If you are unable to provide your audited accounts for the last 2 years, please provide one of the following: a) a statement of the turnover, Profit and Loss Account/Income Statement, Balance Sheet/Statement of Financial Position and Statement of Cash Flow for the most recent year of trading for the Single Nominated organisation. b) a statement of the cash flow forecast for the current year and a bank letter outlining the current cash and credit position showing a projected cash flow (after adjusting for investments in fixed assets). c) alternative means of demonstrating financial status if any of the above are not available (e.g. forecast of turnover for the current year and a statement of funding provided by the owners and/or the bank, charity accruals accounts or an alternative means of demonstrating financial status). If you are a new business, then please submit any accounts that you have and provide a business plan and unaudited trading forecast etc. This document must be signed by your Accountant or Bank. Please also provide a reference from your bank confirming the viability of the business. The Single Nominated Organisation may rely upon the financial standing of another organisation, including a Parent or Group Company or any other organisation willing to act as guarantor for the Single Nominated Organisation ("Other Organisation"), subject to the agreement of the Other Organisation that it will act as guarantor in respect of any financial liabilities of the Single Nominated Organisation. Single Nominated Organisation relying upon the financial standing of another organisation should make this clear and ensure that narratives are consistent and relate to the same entity. The Single Nominated Organisation must also supply evidence of any such agreement with another organisation. The other organisation will be subject to the same criteria and/or financial checks carried out for the Single Nominated Organisation.

	The relevant documents attached must be in the name of the Single Nominated Organisation.
12b	Are you (or any consortium member named above) part of a wider group (e.g. a subsidiary of a holding/parent company)? If "Yes", please provide the name of the organisation and relationship to it and would the parent be willing to provide a guarantee if necessary? If No, state "No" would you be able to obtain a guarantee elsewhere (e.g. from a bank)?
13	Please provide details of any County Court Judgements (CCJs) registered in the last six years on your organisation or any material sub-contractors or consortium members named above and individual company directors, or if you have any legal action pending that could result in a CCJ. If "None", state "None" as your response. If you have CCJs registered in the last six years on your organisation, any material sub-contractors or consortium members named above and individual company directors, or if you have any legal action pending that could result in a CCJ please provide details of action taken together with justifiable reasons and/or other evidence that demonstrates why this does not compromise your organisation's ability to deliver the service.
14	Please self-certify whether you (and any sub-contractors and consortia members named above) already have, or can commit to obtaining prior to the commencement of the contract, the levels of insurance cover indicated below: Employer's (Compulsory) Liability Insurance = £5,000,000 Public Liability Insurance = £10,000,000 Professional Negligence = £5,000,000 Clinical Negligence = £10,000,000 *It is a legal requirement that all companies hold Employer's (Compulsory) Liability Insurance of £5 million as a minimum. Please note this requirement is not applicable to Sole Traders. An answer of "Yes" provides confirmation of self-certification. Note: if you self-certify that you meet the requirements, evidence of this will be requested.
15	Have any insurance or legal claims been upheld against your organisation or any material sub-contractors or consortium members named above within the last 36 months in relation to delivering this or a similar service? If "Yes", please provide details that demonstrate why this would not compromise your ability to deliver this service. If none, please state "None" as your response.
16a	Has your organisation (or any material sub-contractors or consortium members named above), within the last five years, withdrawn from a contract after the contract has been offered (i.e. the organisation has been selected) or awarded (either before or after commencement of the contract)? If "Yes" please provide an explanation of the circumstances and reasons why this

	would not compromise your ability to deliver this service. If "No", please state "None" as your response.
16b	Has your organisation (or any material sub-contractors or consortium members named above), within the last five years, had a contract ended or suspended by the client earlier than the originally intended date? If "Yes" please provide an explanation of the circumstances and reasons why this would not compromise your ability to deliver this service. If "No", please state "None" as your response.
16c	In the last three years, has your organisation or any material sub-contractors or consortium members named above been found guilty of unlawful discrimination by an Employment Tribunal, an Employment Appeal Tribunal or any other court? If "Yes", please provide a summary of the nature of the investigation(s) and an explanation of the outcome to date (if ongoing). If the investigation upheld the complaint against your organisation or any material sub-contractors or consortium members, please explain what action (if any) you have taken to prevent unlawful discrimination from reoccurring. If none, please state "None" as your response. Note: You may be excluded if you are unable to demonstrate to the Commissioner's satisfaction that appropriate remedial action has been taken to prevent similar unlawful discrimination reoccurring.
16d	In the last three years, has your organisation or any material sub-contractors or consortium members named above had a complaint upheld following an investigation by the Equality and Human Rights Commission or its predecessors on grounds of alleged unlawful discrimination? If you have answered “Yes” please provide a summary of the nature of the investigation(s) and an explanation of the outcome to date (if ongoing). If the investigation upheld the complaint against your organisation, please explain what action (if any) you have taken to prevent unlawful discrimination from reoccurring. If none, please state "None" as your response. Note: You may be excluded if you are unable to demonstrate to the Commissioner's satisfaction that appropriate remedial action has been taken to prevent similar unlawful discrimination reoccurring.
16e	In the last three years, has your organisation or any material sub-contractors or consortium members named above been investigated by an Ombudsman, funding body or Commissioner for a case related to unlawful discrimination or breach of the Equality Act 2010, or any members within been found guilty of unlawful discrimination by an Employment Tribunal, an Employment Appeal Tribunal or any other court? If you have answered “Yes” please provide a summary of the nature of the investigation(s) and an explanation of the outcome to date (if ongoing). If the investigation upheld the complaint against your organisation, please explain what action (if any) you (or any material sub-contractors or consortium members named above) have taken to prevent unlawful discrimination from reoccurring.

	If none, please state "None" as your response. Note: You may be excluded if you are unable to demonstrate to the Commissioner's satisfaction that appropriate remedial action has been taken to prevent similar unlawful discrimination reoccurring.
17	Does your organisation (and material sub-contractors and consortium members named above) have a formal Equality & Diversity/Equal Opportunities policy that complies with current legislative requirements, in terms of the following a) Is regularly reviewed and is no more than two years old; b) Covers Workforce and Service Delivery; and c) Includes a Named Accountable Officer. If your organisation does not have an Equality & Diversity/Equal Opportunities policy or has a policy but it does not meet the legislative requirements above, please explain why this is and how you intend to approach and deal with equality and diversity within your organisation.
18a	Has your organisation (or any material sub-contractors or consortium members named above) been convicted of breaching environmental legislation, or had any notice served upon it, in the last three years by any environmental regulator or authority (including local authority)? If "Yes", please provide details of the conviction(s) or notice(s) and details of any remedial action or changes you (or any material sub-contractors or consortium members named above) have made as a result of conviction or notices served. If "No", please state "None" as your response.
18b	If you intend to use sub-contractors, 3rd parties or partners for the delivery of this service, please answer "Yes" to confirm you have processes in place to check whether any of these organisations have been convicted or had a notice served upon them for infringement of environmental legislation? If you have referred to and intend to use sub-contractors, 3rd parties or partners for any element of the delivery of this service but do not have processes in place to check whether any of these organisations have been convicted or had a notice served upon them for infringement of environmental legislation, please answer "No" and provide details of the reasons why and what remedial actions are being taken to rectify this. If you do not intend to use sub-contractors, 3rd parties or partners for the delivery of this service, please answer "Not Applicable". Please note that if you have referred to the use of sub-contractors, 3rd parties or partners for the delivery of this service and have not provided the details above, your application may be rejected.
19a	Please confirm that your organisation and any material sub-contractors or consortium members is taking steps to reduce your GHG Emissions over time and is publicly committed to achieving Net Zero by 2050.
19b	Please confirm that you and any material sub-contractors or consortium members have detailed your environmental management measures by completing and publishing a Carbon Reduction Plan (CRP) which meets the required reporting standard (please refer to PPN 06/21 and appendix A of Document 1) and have provided a link to your most recently published Carbon Reduction Plan or, if you are a new entrant to the market (trading for less than 12 months), please confirm your

	commitment to achieving Net Zero by 2050 and that you will publish a CRP as soon as possible. Where you have confirmed 'Yes' please provide a link to your most recently published Carbon Reduction Plan (CRP).
20a	Has your organisation or any of its Directors or Executive Officers been in receipt of enforcement/remedial orders in relation to the Health and Safety Executive (or equivalent body) in the last 3 years? If "Yes", please provide details of any enforcement/remedial orders served and give details of any remedial action or changes to procedures you have made as a result. The Commissioner will exclude applications that have been in receipt of enforcement/remedial action orders unless you can demonstrate to the Commissioner's satisfaction that appropriate remedial action has been taken to prevent future occurrences or breaches.
20b	Please confirm that all sub-contractors, 3rd parties and partners have in place the relevant Health and Safety Policies, which are compliant with current UK legislation and are not subject to any Enforcement/Remedial Orders in relation to the Health and Safety Executive (or equivalent body) in the last 3 years. If "No", please provide an explanation of your response. The Commissioner will exclude applications that have been in receipt of enforcement/remedial action orders unless the bidder(s) can demonstrate, to the Commissioner's satisfaction, that remedial action has been taken to prevent future occurrences or breaches.
20c	Please provide details of the 'competent person' for provision of health and safety advice within your Single Nominated Organisation as required by The Management of Health & Safety at Work Regulations 1999. Please provide name, job title, details of any health and safety qualifications/training, including experience and knowledge of their particular field. If externally provided, please also include the name and address of the organisation.
21a	Does your organisation (and material sub-contractors and consortium members named above) complete the Data Security and Protection Toolkit (DSPT) (https://www.dsptoolkit.nhs.uk/) and achieve mandatory assertions "standards met" throughout all requirements?
21b	If your Single Nominated Organisations (or any material sub-contractors and consortium members named above) IG assurance is provided by your Parent/Holding company, please attach full contact details and the organisation code for verification and their completed current version of the DSPT and achieved mandatory assertions "standards met" here. If you (or any material sub-contractors or consortium members named above) have not achieved mandatory assertions "standards met" throughout all requirements, please include an IG Action Plan.
22	Please enter yours (and any material sub-contractors and consortium members named above), Data Protection Registration number/s.
23	Does your Single Nominated Organisations have NHSmail or is accredited to NHS Digital's secure email standard DCB1596 (The secure email standard - NHS Digital)? If "Yes" please provide at least one secure NHS email address (*.*@nhs.net) or confirm you are accredited to NHS Digital's secure email standard DCB1596 and

	confirm that you are included on the list of accredited organisations which can be found via the following link: https://digital.nhs.uk/services/nhsmail/the-secure-email-standard (If you are accredited to secure email standard DCB1596 but are not on the list of accredited organisations, please explain why). If "No" please state "No" as your response. Applicants who do not have secure NHSmail accounts (@nhs.net) and are not accredited to NHS Digital's secure email standard DCB1596 may be required to use NHSmail for the delivery of this contract.
24a	Please provide the name and email address for your organisation's Senior Information Risk Owner (SIRO) (and those of any material sub-contractor and consortium member named above) Please note that the details of the organisation's Senior Information Risk Owner (SIRO) (and those of any material sub-contractor and consortium member named above) are included and up to date on the SIRO register: (https://digital.nhs.uk/services/organisation-data-service/update-your-data/registers). Note: Subject to your organisational status (please refer to: https://www.dsptoolkit.nhs.uk/Help/Org-Types), due to the distinct nature of the Caldicott Guardian/Senior Information Risk Owner (SIRO) roles and to prevent any possible conflict of interests the two roles MUST NOT be held by one person.
24b	Please provide the name and email address for your organisation's Caldicott Guardian (and those of any material sub-contractor and consortium member named above). Please note the details of the organisation's Caldicott Guardian (and those of any material sub-contractor's and consortium member named above) are included and up to date on the Caldicott Guardian register: (https://digital.nhs.uk/services/organisation-data-service/update-your-data/registers). Note: Subject to your organisational status (please refer to: https://www.dsptoolkit.nhs.uk/Help/Org-Types), due to the distinct nature of the Caldicott Guardian/Senior Information Risk Owner (SIRO) roles and to prevent any possible conflict of interests the two roles MUST NOT be held by one person.
24c	Please provide the name and email address for your organisation's (and material sub-contractors and consortium members named above) Information Governance Lead.
25a	Will you or any of your partners/subcontractors/3rd Parties be processing or storing Personal Confidential Data (PCD), in relation to this service, outside of the UK? If yes, please provide details of the arrangement and an overview of any outsourcing contracts regarding this with information on compliance with General Data Protection Regulations (GDPR) on transfer of data outside of the UK. If no, please state No as your response.
25b	Please confirm that you (and material sub-contractors and consortium members named above) are GDPR compliant and attach a copy of your DPIA template in relation to having robust Data Protection Impact Assessments (DPIAs) in place at the start of any significant change in processing or identification of any high-risk processing.
25c	Does your organisation (and any third party contracted for the delivery of this service) have access to HSCN hosted services? (Please answer Yes or No)

	If you have answered No, please confirm your organisation's (and any third party contracted for the delivery of this service) willingness to achieve HSCN access and that this will not compromise the Commissioner's planned mobilisation timescales.
26	Does your organisation (and material sub-contractors and consortium members named above) have a Business Continuity/Disaster Recovery Plan? If "Yes", please attach a copy(ies). If "No", please explain why.
27a	Can you confirm that your organisation (and material sub-contractors and consortium members named above) has, or will have prior to the service start date, the proposed workforce policies, strategies, processes and practices to comply with all relevant ACAS Codes of Practice and employment legislation applicable in the UK. Furthermore, that you comply with: 1. NHS Employment Check Standards (July 2010), 2. The UK Code of Practice (CoP) for International Recruitment, 3. CQC Fundamental Standards for Quality & Safety, 4. Disclosure and Barring Service If you have answered "No" please explain why which demonstrates the organisation's ability to deliver the contract is not compromised. Where bidding as a Consortia or intending to use Sub-contractors, 3rd parties and partners, please confirm the above for all members of the Consortium, Sub-contractors, 3rd parties and partners.
27b	Please confirm that all your staff who would work on the service have the appropriate Disclosure and Barring Service Checks.
27c	Please confirm that you understand and accept your organisation's (and material sub-contractors and consortium members named above) responsibilities in relation to TUPE/COSOP which include fulfilling TUPE/COSOP obligations in respect of in-scope staff, liability for any redundancy costs that may occur due to economic, technical or organisational reasons (and not as a direct result of the transfer).
27d	Please confirm that your organisation (and material sub-contractors and consortium members named above) agrees to apply the principles set out in the Cabinet Office Statement of Practice on Workforce Matters in the Public Sector and A Fair Deal for Staff Pensions. https://www.gov.uk/government/publications/fair-deal-guidance https://www.gov.uk/government/publications/fair-deal-policy-and-nhs-pension-scheme
28	Please detail how your staff and other employees' professional registration and professional indemnity is monitored and what action is taken to ensure that these accreditations are kept up to date, encompassing the lead provider and all sub-contractors if applicable